

Deep Research Report on the French START Study and Stimulant Treatment of ADHD With Co-occurring Addictive Disorders

Why the French START study matters

The study you are referring to appears to be the **French START study**, not “STRAT.” The paper is titled *Clinical outcomes of addictive disorders six months after ADHD Diagnosis: Insights from the START study*, and START is expanded by the authors as **Study on the Treatment of ADHD and addiction comorbidity: a ReTropective analysis of medical records**. In the accessible abstract material, it is described as an **observational, retrospective study** using medical records from patients already in addiction treatment, with outcomes assessed **six months after ADHD diagnosis**. The headline result most often cited is that **improvement in addictive disorders was observed in 61.3% of patients**, while **methylphenidate initiation was delayed in 29% of cases**. That immediately tells you this was not a simple “drug switched on, behaviour went down” efficacy trial; it was a **real-world dual-diagnosis service study** in which diagnosis, care engagement, timing of treatment start, and medication all interacted. ¹

That distinction is crucial if you are trying to persuade UK providers. START is valuable not because it proves a clean causal drug effect in the way a placebo-controlled RCT would, but because it suggests that **detecting ADHD inside addiction services and then treating it within routine care can coincide with meaningful early improvement in addictive outcomes**. In other words, START is probably best framed as a **service-pathway signal** rather than a pure pharmacology signal. ²

One more piece of context matters for interpretation. In France, **methylphenidate has historically been the only ADHD medication authorised**, which means the French clinical pathway is structurally different from the UK pathway, where **NICE recommends lisdexamfetamine or methylphenidate as first-line pharmacological treatment for adults with ADHD**. That French prescribing environment may partly explain why START centres so heavily on methylphenidate and why its findings are not automatically transferable to a UK service without adaptation. ³

What can be said with confidence about the START study itself

From the accessible abstract and indexing material, several things are clear. START was a **retrospective chart review**, it studied people **already undergoing addiction treatment**, and it examined addictive-disorder outcomes **six months after ADHD diagnosis**. The accessible abstract also reports that **older age was associated with more favourable outcomes**. What is not clearly visible in the public abstract snippets is the exact **sample size**, the fully specified **primary outcome definition**, the exact **mix of substance and behavioural addictions**, the exact **ADHD diagnostic instruments used in this paper**, or the precise **statistical adjustment strategy** beyond the reported association with older age. ⁴

That missing detail matters. When people quote the “61.3% reduction” or “61.3% improvement” number, they often speak about it as though it were a randomised medication effect size. The accessible sources do **not** support that reading. What they support is this narrower statement: in a real-world cohort of

addiction-treatment patients who were newly diagnosed with ADHD, **61.3% showed improvement in addictive disorders by six months**, and in **29%** the start of methylphenidate was **delayed**, which means the six-month outcome window overlapped routine diagnostic work, therapeutic engagement, and staggered treatment start dates rather than a uniform medication exposure period. ⁵

The most defensible way to present START, therefore, is: **“A French retrospective dual-diagnosis service study found that most patients improved by six months after ADHD diagnosis, within a pathway that included methylphenidate treatment but did not begin medication immediately in all cases.”** That phrasing is evidence-based, persuasive, and much safer than overselling the study as a stand-alone proof that methylphenidate directly caused a 61.3% drop in addictive behaviour. ⁵

How the French operating model appears to have worked

The full methodological “guts” of START are not all visible in open abstract material, but the overlap in authors and institutions lets you reconstruct the **probable operating model** with reasonable confidence from the same French research programme. The START paper shares authors and clinical leadership with the later French **META** protocol, which is a multicentre trial for adults with **ADHD plus at least one addictive disorder**. In META, recruitment happens through **specialised consultations**, ADHD is confirmed using **DIVA-5**, comorbid psychiatric disorders and substance use disorders are assessed with **MINI-S**, and behavioural addictions are assessed with disorder-specific tools such as **NODS**, **YFAS 2.0**, and other structured instruments. Baseline data include demographics, personal and family history, ADHD severity, impulsivity, emotion regulation, self-esteem, and a neuropsychological battery. ⁶

META also shows the operational pattern the French group now believes is feasible in routine-style care. Patients first have ADHD confirmed in a specialist setting, then **methylphenidate is initiated and titrated until dose stabilisation**, and only after stabilisation are they formally enrolled into additional structured intervention. The protocol includes **three research visits**: baseline, short-term follow-up at the end of the active intervention, and medium-term follow-up **six months later**. At follow-up, the team explicitly records **vital signs, side effects, adherence, and misuse of methylphenidate**, as well as all other treatments received for ADHD and addiction. ⁷

Just as importantly, the French team’s model is not “tablet-only.” In META, early sessions are partly centre-based and partly home-based; a **neuropsychologist tracks completion** through software logs and **phones the patient if needed** to encourage continuation. The trial runs across **three university hospitals** and **three non-university public mental health centres**, which indicates the team is consciously designing a model that can work across heterogeneous patients and different service tiers, not just within one highly specialised academic clinic. ⁸

Because START predates META, I cannot state as a fact that START used exactly the same forms and procedures. But it is a reasonable inference that START emerged from the **same French dual-diagnosis network**, with broadly similar principles: **specialist ADHD confirmation inside addiction care, structured comorbidity assessment, stimulant initiation after medical screening, and fixed follow-up windows tied to both ADHD and addictive-outcome monitoring**. That inference is supported by the overlapping author group, the shared institutional base in Nantes and related centres, and the very similar clinical target population. ⁹

What the wider stimulant evidence shows when ADHD and addiction are treated together

The most operationally useful comparator to START is the Swedish trial by **Konstenius and colleagues** in criminal offenders with ADHD and amphetamine dependence. This was a **24-week randomised, double-blind, placebo-controlled trial** in **54 men** recruited from **medium-security prisons** in Sweden. Medication started **within two weeks before release from prison** and continued in community outpatient care with **twice-weekly visits**, including **weekly manual-based cognitive behavioural therapy for relapse**. The dose was not timid: OROS methylphenidate was titrated from **18 mg** up to a maximum of **180 mg/day**. The trial monitored ADHD symptoms, weekly or frequent urine toxicology, craving, blood pressure, pulse, weight, haematology, liver function, and treatment retention. Compared with placebo, the methylphenidate group had **greater ADHD improvement, more drug-negative urines, more amphetamine-negative urines, and better retention in treatment**. ¹⁰

That study is especially important for your purposes because it demonstrates an **operating pattern** that UK providers can recognise: stimulant prescribing was embedded in a tightly managed **transitions-of-care pathway**, with urine testing, relapse-focused psychotherapy, medical monitoring, and very frequent contact. The Swedish investigators even used practical adherence supports such as arranging a **prepaid taxi from the prison gate to the outpatient clinic on the day of release**. This is the opposite of casual prescribing; it is stimulant treatment embedded inside an intensive, highly visible addiction-management structure. ¹¹

A contrasting but equally informative benchmark is the large adolescent multisite trial by **Riggs and colleagues**. This was a **16-week, randomised controlled multisite trial** of **OROS methylphenidate plus CBT versus placebo plus CBT** in **303 adolescents aged 13–18** with DSM-IV ADHD and substance use disorders. The primary outcomes were clinician-rated ADHD symptoms and self-reported days of substance use; secondary outcomes included parent ratings and **weekly urine drug screens**. The main result was negative on the primary endpoints: there was **no significant between-group difference** in reduction in ADHD symptoms or days of substance use. However, some secondary outcomes favoured OROS methylphenidate, including **better parent-rated ADHD improvement** and **more negative urine drug screens**. ¹²

That pattern is very useful when reading START. It shows that in dual-diagnosis populations, once you add a strong psychotherapy platform such as individual CBT, the incremental drug-placebo signal can become harder to detect on broad primary outcomes even when meaningful improvements are still visible on secondary or objective measures. In other words, **negative or mixed stimulant trials do not necessarily mean “ADHD treatment is irrelevant”; they may mean the background psychosocial treatment is already powerful and the signal becomes distributed across multiple outcomes**. ¹³

Behavioural-addiction evidence is more limited, but it points in the same direction as START. In a **naturalistic prospective** study, **37 consecutive adult ADHD outpatients** were assessed at baseline and again after **one year of flexible-dose methylphenidate treatment**. The investigators found significant reductions in **internet addiction, shopping addiction, food addiction, and sex addiction** symptoms, and the proportion of patients with at least one behavioural addiction fell from **51.4% to 35.1%**. That is not a dual-diagnosis addiction-service trial like START, but it is highly relevant because it suggests that stimulant treatment of ADHD may coincide with measurable improvement in **behavioural** as well as substance-related addictive phenomena. ¹⁴

For lisdexamfetamine, the evidence is thinner when current ADHD and current addiction are both present. A post-hoc analysis by **Kollins and colleagues** examined adults with ADHD and a **history of**

substance use disorder inside a randomised, double-blind, placebo-controlled lisdexamfetamine trial. The subgroup with SUD history was small, only **17 participants**, and current comorbid SUD was excluded, so it is **not** a direct analogue of START. Even so, the study found that lisdexamfetamine's ADHD efficacy and tolerability in that subgroup were similar to the broader adult ADHD sample. That tells you lisdexamfetamine is plausible in people with a prior addiction history, but it does **not** answer the question of how best to use it in an active ADHD-addiction pathway. ¹⁵

A separate lisdexamfetamine proof-of-concept trial in **43 treatment-seeking adults with cocaine dependence**, conducted at the **University of Minnesota Ambulatory Research Center**, is helpful on operating model even though it excluded ADHD. It used a **1-week intake, 1-week medication induction, 14-week treatment**, and **1-week reduction** period. Participants were recruited through local media, screened by telephone, evaluated in person, and monitored with weekly craving measures, side-effect checks, blood pressure, heart rate, weight, adherence markers, and urine toxicology. In the randomised sample, lisdexamfetamine did **not** significantly reduce cocaine use, but it **did** reduce cocaine craving. The investigators emphasised both the need for structured monitoring and the fact that the 70 mg dose was constrained by ADHD prescribing limits, which may have limited efficacy in a stimulant-dependent population. ¹⁶

Finally, the broader evidence base remains mixed. A 2023 clinician review concluded that stimulants may help **both ADHD symptoms and some cocaine- or amphetamine-related outcomes** in selected patients, but results across trials are inconsistent. A 2024 evidence review for the US VA concluded that, among people with co-occurring ADHD and SUD, evidence for stimulant effects on abstinence, adherence, and SUD symptom change remains **low confidence**, with many studies being old, small, and centred on stimulant-use disorders rather than the full spread of addictions seen in practice. That means START is interesting precisely because it sits closer to routine dual-diagnosis care than most earlier RCTs. ¹⁷

The caveats you will need when discussing the 61.3% figure

The biggest caveat is causality. START was a **retrospective observational** study, not a prospective randomised comparison. Without a control group, the reported improvement rate could reflect several things at once: the effect of correctly diagnosing ADHD, the effect of beginning stimulant treatment, the effect of increased therapeutic engagement after a new explanatory diagnosis, the effect of ongoing addiction treatment already in place, and the ordinary improvement sometimes seen over time in treatment cohorts. The fact that methylphenidate initiation was **delayed in 29% of cases** makes that caveat even stronger. ⁵

The second caveat is measurement transparency. In the publicly accessible abstract snippets, START does not clearly disclose the **sample size**, the exact **operational definition of “improvement in addictive disorders,”** or whether outcomes were standardised, clinician-rated, patient-reported, or extracted from routine clinical notes. For a commissioner or provider audience, that means START is strongest as a **real-world proof of concept** and weaker as a **regulatory-style efficacy dataset**. ¹⁸

The third caveat is service context. France's adult ADHD pharmacotherapy ecosystem differs from the UK's because methylphenidate has had a uniquely central role there. In the UK, providers will rightly ask whether a similar pathway should use **methylphenidate, lisdexamfetamine, or both**, and whether elements should sit in specialist ADHD clinics, addiction teams, or integrated neighbourhood neurodevelopmental pathways. START does not solve those commissioning questions on its own. NICE and the NHS England ADHD Taskforce both point towards a more **integrated, cross-sector model**, rather than a narrow single-specialty solution. ¹⁹

The fourth caveat is external validity. The strongest comparative methylphenidate trial in this space, the Swedish prison-release study, succeeded inside a highly controlled environment with **supervised abstinence, urine testing, structured psychotherapy, adherence logistics, and frequent visits**. When UK providers ask whether START can be replicated, the honest answer is: **probably not by copying the prescription alone**. What needs to be replicated is the **whole operating model** around the prescription. ²⁰

What this means for an OTOS-style UK replication

If your goal is to convince UK service providers to take part in a similar model, the most compelling argument is not “France proved methylphenidate fixes addiction.” The more persuasive, better-evidenced argument is this: **when ADHD is actively identified within addiction services, stimulant treatment is initiated in a structured and medically supervised way, and outcomes are tracked with the same seriousness as addiction outcomes, both functioning and addictive behaviour can improve in clinically important ways**. START supports that proposition at service level; the Swedish OROS methylphenidate study shows it under tighter experimental conditions; the Grassi behavioural-addiction study extends it into behavioural outcomes; and expert consensus supports **integrated** rather than sequential management. ²¹

For a UK model, six design features look especially well supported. First, **specialist confirmation of ADHD inside or immediately adjacent to addiction care**, rather than bouncing patients between services. Second, **baseline medical screening** before stimulant initiation, including cardiovascular checks and review of psychiatric risk. Third, a preference for **long-acting formulations** where misuse or diversion is a concern. Fourth, **fixed follow-up windows** with explicit recording of ADHD symptoms, functional impairment, substance use or behavioural addiction outcomes, adverse events, adherence, and misuse. Fifth, **multimodal treatment**, so medication sits alongside psychological work rather than replacing it. Sixth, strong **communication loops** across the prescriber, addiction clinician, psychologist or neuropsychologist, GP, and—where relevant—criminal justice or probation services. Those features are either explicit in the French META protocol, explicit in NICE and UK shared-care guidance, or strongly reflected in the Swedish and adolescent co-treatment trials. ²²

There is also a strategic advantage in aligning this with current NHS thinking. The NHS England ADHD Taskforce has explicitly argued that **ADHD is not the remit of health alone**, and that support, data, and service planning need to span multiple agencies and settings. Its later report also argues for **digitisation** and embedding ADHD assessment and care more locally, including within neighbourhood and integrated models. That makes a UK OTOS-style pathway easier to pitch if it is framed as a **cross-sector, outcomes-driven dual-diagnosis model**, not merely as a request for more stimulant prescribing. ²³

The practical message to providers is therefore straightforward. START is promising, but its real lesson is operational: **diagnose ADHD properly, treat it early, stabilise medication, monitor addiction and functioning in parallel, and build frequent, shared, transparent follow-up into the pathway**. If you present the 61.3% figure inside that broader, cautious, evidence-based narrative, it becomes much more convincing than if it is used as a lone headline statistic. ²⁴

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³ <https://www.frontiersin.org/journals/psychiatry/articles/10.3389/fpsyt.2022.957242/full>
<https://www.frontiersin.org/journals/psychiatry/articles/10.3389/fpsyt.2022.957242/full>

6 9 <https://pubmed.ncbi.nlm.nih.gov/41916643>

<https://pubmed.ncbi.nlm.nih.gov/41916643>

7 22 <https://www.researchgate.net/publication/>

403374801_Randomised_controlled_trial_on_the_efficacy_of_adding_cognitive_remediation_therapy_to_methylphenidate_in

https://www.researchgate.net/publication/403374801_Randomised_controlled_trial_on_the_efficacy_of_adding_cognitive_remediation_therapy_to_n

8 <https://bmjopen-bmj-com.ezproxy.u-pec.fr/content/bmjopen/16/3/e111698.draft-revisions.pdf>

<https://bmjopen-bmj-com.ezproxy.u-pec.fr/content/bmjopen/16/3/e111698.draft-revisions.pdf>

10 11 20 <https://www.diva-portal.org/smash/get/diva2%3A698734/FULLTEXT01.pdf>

<https://www.diva-portal.org/smash/get/diva2%3A698734/FULLTEXT01.pdf>

12 13 <https://ctnlibrary.org/document/710/>

<https://ctnlibrary.org/document/710/>

14 [https://www.researchgate.net/publication/385824717_Long-](https://www.researchgate.net/publication/385824717_Long-term_changes_on_behavioral_addictions_symptoms_among_adults_with_attention_deficit_hyperactivity_disorder_treated_v)

term_changes_on_behavioral_addictions_symptoms_among_adults_with_attention_deficit_hyperactivity_disorder_treated_v

[https://www.researchgate.net/publication/385824717_Long-](https://www.researchgate.net/publication/385824717_Long-term_changes_on_behavioral_addictions_symptoms_among_adults_with_attention_deficit_hyperactivity_disorder_treated_v)

term_changes_on_behavioral_addictions_symptoms_among_adults_with_attention_deficit_hyperactivity_disorder_treated_with_methylphenidate

15 <https://pubmed.ncbi.nlm.nih.gov/21468295/>

<https://pubmed.ncbi.nlm.nih.gov/21468295/>

16 [https://www.sbcounty.gov/uploads/DBH/2023/10/](https://www.sbcounty.gov/uploads/DBH/2023/10/Mooney%20et%20al%20cocaine%20and%20Vyvanse.pdf)

Mooney%20et%20al%20cocaine%20and%20Vyvanse.pdf

<https://www.sbcounty.gov/uploads/DBH/2023/10/Mooney%20et%20al%20cocaine%20and%20Vyvanse.pdf>

17 <https://pmc.ncbi.nlm.nih.gov/articles/PMC10179386/>

<https://pmc.ncbi.nlm.nih.gov/articles/PMC10179386/>

19 <https://www.nice.org.uk/guidance/ng87/chapter/recommendations>

<https://www.nice.org.uk/guidance/ng87/chapter/recommendations>

23 <https://www.england.nhs.uk/wp-content/uploads/2025/06/PRN02031-interim-report-of-the-independent-adhd-taskforce-part-1.pdf>

<https://www.england.nhs.uk/wp-content/uploads/2025/06/PRN02031-interim-report-of-the-independent-adhd-taskforce-part-1.pdf>